

SECTION II

Conceptualizing Addiction

ATTACHMENT WOUNDS, TRAUMA, AND THE ADDICTIVE SYSTEM

When children can bask in the warmth of stable, affectionate, and attentive caregivers, they feel safe and learn to trust themselves and others as they venture out into the world. They trust that they can express all their feelings, including silliness, grief, anger, and more. Kids who are secure and safe also feel joy and delight in the power and energy of their body—they touch everything in sight; strengthen their muscles through play; splash in bathtubs; move through the world on scooters, bicycles, and bare feet; and tenderly reach for the hand of a younger sibling or a friend.

In the ideal childhood, kids know that they're free to make mistakes and miss the mark as they learn. They know help is nearby if consequences arise and they will receive gentle guidance when they need to make a repair. Their caregivers protect them from the dangerous conditions and people they meet along the way. Their system and the system around them are in balance. Their managers discover the joy, pride, and satisfaction of accomplishment and growth. Their firefighters develop pleasurable, healthy activities that allow them to relax and pursue new adventures. Their vulnerable parts are open-hearted, loving, and receptive to love. They feel secure, loved, and free to be fully embodied. They can flourish.

However, life is rarely ideal. Obstacles pop up along the way. While emotional and physical pain are part of life, many clients with addictive behaviors have experienced more pain than they could bear. Some clients grew up in the midst of excruciating family dynamics, exploited or neglected by adults who they feared and reviled but also longed for and sometimes—confoundingly—loved. Since young children read their emotions as important information about *themselves*, not about others in their environment, those who experience neglect or abuse are especially likely to feel personally responsible for their caretaker's behavior.

But clients can grow up in more stable environments and, still, for one reason or another, develop burdensome beliefs about their value and lovability. Some clients may struggle as a result of their temperament or a specific learning disability. Others get thrown off balance by painful relationship ruptures later in life, or they collapse when connections fracture due to a divorce or an untimely death. Still others have physical challenges or chronic illness to contend with. All of these kinds of experiences can burden vulnerable parts and summon protectors who offer relief. Life is wounding, and a person may engage in compulsive practices at some point along the way for innumerable reasons. Whatever the cause, IFS therapists build a collaborative, nonjudgmental connection that helps clients heal their wounded parts and free their protectors.

THE SOCIAL CONTEXT OF ADDICTION: POLARIZATIONS WRIT LARGE

Not only does the family system deeply affect the development of a child's inner system, but the family system also nests within larger social systems that are profoundly influential. This includes systems at the neighborhood and community level, such as schools and religious organizations, as well as local, state, and federal government systems. These systems often contain a variety of polarizations that exacerbate the tension between managers and firefighters and contribute to a social

context that exacerbates addiction. This is particularly the case in American culture and its institutions, which contain numerous polarities that illustrate the tension between constraint and compulsive disinhibition, between harsh moralizing and encouraged indulgence.

The economy in the United States, for example, is built on compulsive conspicuous consumption, yet compulsive practices are viewed as a moral failing. Religious and governing institutions police sexuality, female bodies, and sexual preferences while mainstream advertising sexualizes just about everything, making especially liberal use of the female body. Some psychoactive substances are banned and vilified, while others—with equal or greater potential for destructive use—are legal. Similarly, corporations market unhealthy, fattening foods and fight to keep them placed in schools for children, while the mainstream media celebrates anorexic bodies and sermonizes that thinner is better. Manager parts absorb and adopt these confusingly contradictory views at an early age—in the family, at school, in places of worship, in the media, and in an individual's interface with government institutions.

The polarity between inhibition and disinhibition is writ particularly large in the so-called war on drugs. Here, we find a strong and steady appetite for substance use across all demographic groups (Hart, 2021), which contrasts sharply with draconian government prohibitions that were designed to systematically disenfranchise Black, Indigenous, Latinx, and Asian communities, as well as other immigrants and people of color, for drug use (Hari, 2015; Hart, 2021). Government institutions have historically attacked recreational drug use among Black males and incarcerated them at five times the rate of their White counterparts, who were using recreational drugs at very similar rates. In the same context, pharmaceutical companies have, in recent years, made enormous profits by openly pedaling patently false information about prescribed opiates, leading to about 50,000 yearly overdose deaths in the United States by 2019.

And this is only a small part of the picture. We see this epidemic of addiction-related deaths in the context of larger epidemics that relate to disinhibition. Each year in the United States, while approximately 95,000 people die alcohol-related deaths and more than 100,000 die drug-related deaths, an additional 300,000 die from various causes related to obesity, and 480,000 die from the use of tobacco. However, millions of people eat, drink alcohol, gamble occasionally, or use drugs recreationally without causing themselves any major difficulties (Hart, 2021). The fatalities that result from compulsive practices are primarily, in our view, evidence of unaddressed suffering. This suffering can arise from large-scale issues (like institutional racism, patriarchy, and the entrenched materialism of a consumer-based economy) or personal histories of trauma and loss (Felitti, 2004; Hari, 2015; Menakem, 2017; Szalavitz, 2016; Schwartz & Sweezy, 2020).

Although individual psychotherapy can certainly not overhaul social ills, we continue to work to repair the individual lives of our clients. If a client lives in dangerous circumstances or engages in criminalized activities, we can guide them to hear the stories of their firefighter parts and the impact of their childhood environment. If their circumstances at home are dangerous, we can validate their reality and help them access services and reclaim parts they have exiled for personal, social, or political reasons. Validation is an antidote to shaming experiences and the remedy for internalized mistreatment, but it can't only emanate from us. A client must get validation from *within* by detaching from critical protectors, listening to their intentions with compassion and understanding, and offering a safe refuge to wounded parts.

POLARIZATIONS BETWEEN PROTECTORS

Given the various polarities we see at play in the context of the family and larger social systems, it should come as no surprise that polarizations between protective parts are the norm in addictive processes. These oppositions form when managers and firefighters devote themselves to contrasting strategies of protecting an internal system that is chronically threatened by exposure to frightening

memories and painful feelings. Both teams of protectors are constantly striving to keep the system from being overwhelmed by exiled parts. While manager parts help the client function, firefighter parts use substances and other forms of soothing or distraction to keep the feelings of exiles at bay. Alone, untended, and often denigrated as too weak or too innocent, exiles get caught in the crossfire. Their unmet needs escalate, they feel ever more hopeless, and so it goes, sometimes for years, before the individual seeks or gets access to therapy.

THE POLARIZATION CYCLE

1. Fragile, underlying exiles who feel deficient, inadequate, and abandoned—for any number of reasons, both historical and present-day—activate due to external interactions or challenges in the client’s life.
2. Managers ignore or try to contain these vulnerable parts by getting busy, concentrating on tasks or the needs of others, keeping the client in their head, or using criticism to goad the exile into improving and becoming acceptable.
3. Noting the exile’s distress and shamefulness, firefighters take over, using substances and various practices (*whatever it takes!*) to mask or medicate emotional pain.
4. Exiles feel sick, degraded, fearful, and isolated.
5. Managers mobilize again:
 - a. Task managers frantically mobilize to get operations back on track in hopes of regaining fleeting control and self-respect.
 - b. Critical, moralizing managers attack, vilify, and shame firefighters for their repeated transgressions.
6. Firefighters return to the addictive practice again (*something, anything!*) to further medicate the pain, block out the shame, and deny consequences.
7. Vulnerable exiles, unsought and unwanted, feel abandoned again, which reinforces their sense of being hopeless and unlovable. The cycle continues.

Since all of this occurs in a cycle, we prefer the term *addictive process* instead of *addiction*. Referring to addiction as a process also implies the developmental aspects of compulsive using, since in a chaotic environment, the addictive process can begin early (Lewis, 2015; Szalavitz, 2016). Here, a child may oscillate between manager parts who crave control and good behavior and firefighter parts who crave diversion, escape, and pain relief. For example, a child who feels compelled to take care of the wayward adults in their life might vacillate between a part who excels at sports and a part who is

always dieting. They may fluctuate between a part who strives for perfect grades and a part who smokes cigarettes and shoplifts. These activities distract from, but do not help, the child's deprived young parts who need parenting. As exiles feel more desperate and the child's firefighter team gets angrier, more desperate, and more extreme, the balance between inhibition and disinhibition becomes ever more precarious. This is an addictive process.

When a client's vulnerable parts have not been as deeply wounded, the inner dynamic is milder but still present. They may wind up feeling a little stuck, a little unsuccessful. The client may not be as patient as they'd like with their partners or kids. In turn, their firefighters go to work to distract and soothe. They spend too much time lying around playing video games or scrolling through social media. Meanwhile, their managers nag and scold these screen-loving, couch-potato firefighter parts to close that laptop, get off the sofa, and be a little nicer today! This is not a major inner battle, but it is a polarity.

Polarities escalate in response to emotional pain that stems from an accumulation of negative experiences that harsh managers echo internally. Some clients report a long developmental trajectory for their addictive process and have had treatment before. Some arrive at therapy for the first time, feeling spurred to action by some threat, like job loss, after many years of trying to control their addictive process. In contrast, some clients report that an addictive behavior only became a problem after a recent stressor. Widowed seniors, for example, may start gambling away their homes and pension checks after losing their partner, illustrating a very recent need for relief from isolation and grief.

Once the addictive process (*I have to do it! I have to stop doing it!*) becomes extreme and chronic, it can't be resolved with simple negotiation. The antipathy and lack of trust between the two sides runs too deep. Therefore, rather than soliciting one side for discussion, we guide the client to validate the good intentions of both sides without encouraging or endorsing the very extreme behaviors on opposite limits of the spectrum. For example, it's essential to function, grow, and be responsible—but it's also essential to feel carefree, rested, and satisfied. We show managers and firefighters how they are linked, and we assure them that the client needs *both* teams for their complete well-being. As they begin to feel validated and safer, they will become more cooperative.

IS PSYCHOTHERAPY ENOUGH?

For clients who struggle with addictive processes that involve high-risk, self-harming behavior, psychotherapy isn't always enough. Some people need intense outpatient treatment or hospitalization. Peer support groups work wonders for many, but not all. Some will benefit from medication or the support of other professionals, such as a nutritionist or an acupuncturist. Some could use an adjunctive treatment, like couple therapy, art therapy, or a meditation group. Some won't be willing to use these kinds of options, and some won't have access to them. We can expect progress to go in fits (which we call recurrences) and starts (which is called progress). But whether an individual seeks help in a community mental health center, residential treatment, or private practice, psychotherapy is a central resource.

So let's drill down on this question: *What can we do in the therapy hour?* We can assert that the client's addictive process is fueled by the need to relieve emotional pain, we can offer clarity that normalizes their inner battles and helps to organize chaotic thinking, and we can show them how to get lasting relief by safely healing old wounds.